

Children with Medical Complexity - Factoid Sheet

Week 7 • Holistic Patient Care • Kapoor • memorize-the-framework, not trivia

CMC = chronic conditions + functional limitations + high health-care use + substantial family/service needs.

Faculty objectives

1. Manage complex comorbidities
Baseline, devices, medications, multidisciplinary care, safe emergency/discharge planning.

2-3. Unique needs + family-centered care
Caregiver/sibling burden, social/financial barriers, technology dependence, family partnership and shared decisions.

What makes these patients different clinically?

Domain	Ask
Baseline	What is normal for this child: vitals, oxygen/ventilation, mental status, feeding, mobility, communication?
Technology	Trach/vent, oxygen, G-tube, pumps, catheters, mobility devices - what fails and what is the backup?
Medications	Polypharmacy, time-critical drugs, rescue meds, interactions, actual home availability.
Care team	PCP, tertiary specialists, therapists, school, home nursing, pharmacy/equipment supplier.
Family capacity	Work, finances, transportation, siblings, sleep, health literacy, caregiver stress, home nursing.

Parent perspective: LISTEN • TEACH • CALM.

Outpatient + emergency care

- Same chief complaint can carry very different risk depending on baseline and technology dependence.
- Parents often know the earliest deviations from baseline.
- Emergency-plan challenges: fragmented care, medication availability, technology dependence.
- Useful solutions in the deck: systematic ED care plan, ready-to-go access kit, respiratory-care resources, care-coordination meetings.

Family burden

Assess **caregiver mental health, siblings, financial impact, employment, social isolation, and logistics**. A medically correct plan that the family cannot implement is not a safe plan.

Transitions, Inpatient Care & Palliative Care

Inpatient framework

Whole child + multidisciplinary team + family-centered care.

Whole child	Do not reduce the patient to the admitting diagnosis; include nutrition, development, comfort, communication, mobility, school, psychosocial needs.
Multidisciplinary	Prevent contradictory plans; clarify who owns each problem.
Family-centered	Share information, use caregiver expertise, teach, and make decisions collaboratively.

Transition home

Discharge question: Can the family safely perform the care with the equipment, medications, nursing, supplies, transportation, training, and backup plan that are actually available?

- Home nursing may be limited and varies by state/coverage.
- Caregiver training in equipment and medication administration is essential.
- Build routines, organize supplies, reduce isolation, and account for other family members.

Palliative care

What it provides • Symptom management • Psychosocial support • Communication • Care coordination	Also • Shared decision-making • Spiritual support • Quality-of-life focus • Can coexist with disease-directed treatment
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7-step clinical framework

1	Baseline?	Know what "normal" is for this child.
2	Technology?	Devices, failure modes, backups.
3	Medications?	Time-critical drugs + reconciliation.
4	Caregiver expertise?	Listen to the people who know the child.
5	Coordination?	Who needs to talk to whom today?
6	Feasible?	Can this family actually execute the plan?
7	Backup?	Warning signs, emergency steps, contact plan.

Last 5 minutes: Complexity is a care-system issue. Know baseline + devices + meds. Partner with caregivers. Coordinate teams. Make transitions explicit. Check family capacity. Palliative care supports quality of life and decisions - it is not synonymous with end-of-life care.

Source: Kapoor 30-slide "Caring for Children with Medical Complexity" session. Separate HPC resource; not counted as a tenth core Hematology lecture.